

DOCUMENT 1: NEW PATIENT CONSULTATION

DATE: 30-Aug-2021

PATIENT: EDWARDS, Dennis (AML053)

REASON FOR CONSULT: Newly diagnosed AML.

HPI: Mr. Edwards is a 55-year-old male, a partner at a law firm, who was in his usual state of excellent health until 3 weeks ago when he developed debilitating fatigue, drenching night sweats, and noticed a new rash on his shins. Presented to ED where CBC showed WBC 55.4 K/uL with 80% blasts. Admitted for workup.

DIAGNOSTICS: Bone marrow biopsy (28-Aug-2021) confirms AML. Cytogenetics are normal (46,XY). Molecular testing is positive for a **FLT3-ITD mutation** at a high allelic ratio of 0.78. NPM1 is wild-type. This classifies him as **ELN 2022**

Intermediate-risk.

ASSESSMENT: Fit 55 y/o male with hyperleukocytic, FLT3-ITD positive AML.

PLAN:

1. Initiate hydroxyurea immediately for cytoreduction.
 2. Recommend against standard "7+3" induction due to high risk of tumor lysis syndrome (TLS) and the availability of superior targeted therapy.
 3. Recommend first-line treatment with oral FLT3 inhibitor **Gilteritinib (Xospata)**. Discussed risks/benefits, including cytopenias, LFT abnormalities, and QTc prolongation. Patient is in agreement.
 4. Plan to start Gilteritinib 120 mg daily on 04-Sep-2021 after insurance approval. Will monitor labs closely for TLS.
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DOCUMENT 2: OUTPATIENT FOLLOW-UP NOTE

DATE: 15-Mar-2022 (6 months on therapy)

S: Patient feels "completely normal." He is back to working full-time. No fatigue, bleeding, or infectious complaints.

O: Vitals stable. Exam unremarkable.

Labs: CBC is excellent: WBC 4.2, ANC 2.5, Hgb 13.8, Plt 198. LFTs normal. EKG shows QTc of 445ms (stable).

BMBx (from 01-Mar-22): Morphologic and flow cytometric CR. Importantly, MRD testing by NGS for his specific FLT3-ITD mutation is **negative**.

A/P: Mr. Edwards is in a deep molecular remission on Gilteritinib. Excellent response. Continue Gilteritinib 120 mg daily. RTC in 3 months.

DOCUMENT 3: URGENT CARE CLINIC NOTE

DATE: 02-Feb-2023 (17 months on therapy)

S: Patient presents with a 2-week history of recurrent gingival bleeding, profound fatigue, and bone pain in his sternum. States he feels "just like I did when this all started."

O: Exam shows pallor, extensive petechiae on arms and legs. Gums are swollen and friable.

Labs (STAT): WBC 1.9, Hgb 7.2, Plt 12. Peripheral smear reviewed manually: 40% circulating blasts are seen.

A/P: Acute relapse of FLT3-ITD positive AML. He has developed resistance to Gilteritinib. Admitted directly to the inpatient hematology service for management of severe pancytopenia and to discuss salvage therapy options.

PLAN:

1. Admit to Oncology ward.
2. Transfuse pRBCs and platelets STAT.
3. Hold Gilteritinib.
4. Schedule family meeting to discuss prognosis and options (intensive salvage chemotherapy vs. Venetoclax-based regimen vs. clinical trial).

DOCUMENT 4: BEREAVEMENT LETTER TO PCP

TO: Dr. Susan Reid, MD

FROM: University Cancer Institute

DATE: 15-Jul-2023

RE: In Memory of Dennis Edwards, DOB 03-Jun-1966

Dear Dr. Reid,

It is with great sadness that I write to inform you of the passing of our mutual patient, Mr. Dennis Edwards. He passed away on **July 10, 2023**.

Following his relapse in February, a repeat bone marrow biopsy confirmed refractory disease and also revealed the acquisition of a new TP53 mutation, a known mechanism of resistance to FLT3 inhibitors. We had a long discussion with him and his family, and he elected to enroll in a Phase II clinical trial. Unfortunately, he did not respond to the investigational agent and developed progressive disease complicated by fungal pneumonia. He was transitioned to inpatient hospice care and died peacefully.

Dennis was a remarkable man who faced his illness with incredible courage. We thank you for the opportunity to have participated in his care.

Sincerely,

Dr. Eleanor Vance, MD